

Psychological support can be distinguished from psychotherapy: Clarifications for future empirical work[☆]

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ABSTRACT

Classic psychedelic studies involve interpersonal interactions with participants before, during, and after the acute effects of psychedelics. In the field of psychedelic research, there is currently a debate about whether this interpersonal interaction is most accurately considered “psychological support,” “psychotherapy,” or something else. Proponents of the psychological support label describe the interpersonal interaction in most classic psychedelic studies as aimed at safety, while proponents of the psychotherapy label argue that the interpersonal interaction involves intervening on psychotherapeutic processes that not only support safety, but also contribute to treatment efficacy. However, we believe that the debate has so far been largely unproductive due to a lack of shared definitions or criteria for the relevant interpersonal interactions. Here, we propose definitions and criteria for two viable forms of interpersonal interaction in psychedelic administration: one directed solely towards safety (psychological support) and one directed towards both safety and efficacy (psychotherapy). We then trial our proposed criteria on a sample of 11 published classic psychedelic clinical trials. Our procedure categorized 10 out of 11 studies as being more or less consistent with psychological support or psychotherapy, but achieved clear distinctions in only 4 out of 11 studies. Given these findings, we argue that higher quality data on the interpersonal interaction is needed to clearly distinguish between psychological support and psychotherapy. We conclude by suggesting ways that our criteria can be used in the future to empirically test the relative safety and efficacy of psychological support vs. psychotherapy in psychedelic medicine.

1. Introduction

Contemporary research on classic psychedelics, such as psilocybin and LSD, has primarily used study designs that administer the psychedelic substance along with a concurrent interpersonal interaction involving contact between the participant and the facilitators before, during, and after the psychedelic experience¹ (e.g. [1,2,3,4]). In many psychedelic studies, these interpersonal interactions occur during the preparation phase (i.e., before the participant receives the drug), during

drug administration, and during the integration phase (i.e., after acute effects of the drug have subsided) [5,6]. Though almost all psychedelic studies thus far have administered psychedelics with an interpersonal interaction, the interpersonal interaction itself is heterogeneous across studies, imprecisely defined, inconsistently labeled and applied, rarely evaluated for adherence/fidelity, and poorly understood [7,5,8,9]. A debate has emerged about whether the interpersonal interaction paired with psychedelic administration is intended solely to maintain safety or is intended to maintain safety *and* enhance efficacy. Although the aim of

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¹ As our paper discusses the distinction between psychotherapy and psychological support, we have opted for “interpersonal interaction” as an attempted neutral term to describe the interpersonal interactions that occur within psychedelic treatments, to avoid reference to either approach.

enhancing efficacy through the use of the interpersonal interaction is eminently valuable from a clinical perspective, an efficacious interpersonal interaction challenges scientists' and regulators' ability to interpret the independent causal efficacy of psychedelic drugs.²

In a recent commentary titled “Must psilocybin always ‘assist psychotherapy’?”, Goodwin and colleagues argue that psychotherapy might not be needed to obtain therapeutic benefit from psychedelic drugs, citing recent psilocybin studies in which the interpersonal interaction was primarily directed at safety, yet treatment effect sizes were comparable to studies in which the interpersonal interaction was intended to function as a psychotherapy. Goodwin et al. go on to hypothesize that psilocybin is likely efficacious with nothing more than the provision of “psychological support,” an interpersonal interaction that they argue is *directed solely at the safety of the participant* before, during, and after psychedelic administration. They argue that in contrast to psychotherapy, psychological support is a less-intensive and less-directive interpersonal interaction that can be offered in a standardized and consistent way [10,11]. Goodwin et al.'s proposal is thus distinct from the “psychedelic-assisted therapy” (PAT) model, where the drug is seen as a catalyst for explicitly psychotherapeutic treatment, or as an enhancement to an already efficacious psychotherapeutic process [10,11]. Goodwin and colleagues' arguments are not hypothetical: a number of psychedelic studies, including the COMP360 trial³ of which Goodwin is one of the authors, have applied the label of “psychological support” rather than “psychotherapy” to their interpersonal interaction. However, as discussed below, there is disagreement as to whether the interpersonal interactions labeled as psychological support in the extant literature are conceptually or practically distinct from those labeled as psychotherapy.

Goodwin et al.'s article prompted critical commentary from several researchers in the field [12,13,14,15,16,17]. These responses raised a variety of issues, including: 1) a lack of empirical evidence for the arguments made by Goodwin et al. and 2) concerns about safety and ethical issues if the psychotherapy component was to be removed.

Regarding the perceived lack of empirical evidence, both Deckel et al. and Bogenschutz argue that there is insufficient empirical support at present for the claim that therapeutic effects can be attributed solely to the drug itself. Earleywine and colleagues note that only a clinical trial comparing both approaches (psychotherapy vs. psychological support) will be able to answer this question (although they express concerns about the potential ethical issues in doing so) [13,14,16]. Addressing safety, O'Donnell and colleagues write that Goodwin and colleagues ignored data on the effect of the therapeutic alliance in mediating the anti-depressant effects of psilocybin and that further reducing psychotherapy in psychedelic clinical trials could lead to a greater risk of adverse outcomes for participants [15]. Similarly, Schenberg and colleagues argue that psychotherapy plays a crucial role in optimizing both safety and efficacy in psychedelic clinical trials, and the suggestion that effectiveness is solely attributable to drug effects may result in unintended harms to patients [17]. Alpert et al. also

disagree with Goodwin and colleagues as to the role of the drug effect, going so far as to argue that the psychedelic “drug effect per se is likely illusory” [12]. All commentaries on the Goodwin et al. article argue that it may be premature to remove psychotherapy from psychedelic treatments [12,13,14,15,16,17].

In their response to these commentaries, Goodwin and colleagues emphasize that they are not necessarily arguing for a reduction in the supportiveness of the interpersonal interaction; rather, they argue that the interpersonal interaction should not be directed towards maintaining or enhancing efficacy, but should rather limit itself to a focus on safety alone. In addition, Goodwin et al. note that most of the critical responses to their commentary reference midomafetamine (MDMA)-assisted therapy for PTSD, rather than the focus of their article, which was on psilocybin and other classic psychedelics. This distinction is important, as MDMA is more routinely paired with intensive psychotherapy, likely because of the empathogenic/entactogenic effects that it elicits and the exposure-based paradigm that likely accounts for its benefits in PTSD. Classic psychedelics, on the other hand, tend to elicit a more inward directed experience that generally requires far fewer interactions between facilitators and participants [10,11]. They additionally debate O'Donnell et al. [15] about what constitutes a “bona fide” psychotherapy and disagree about whether the interpersonal interaction offered in recent psilocybin studies is best characterized as psychological support or psychotherapy [10,11].

This point raised about drug type is important. While ketamine administration often involves almost no psychosocial support, MDMA for PTSD involves a high degree of support and psychotherapy. The question here is where psilocybin and other classic psychedelics fall on this spectrum (see Fig. 1).

With the goal of resolving the debate over what constitutes bona fide psychotherapy vs. what constitutes psychological support in psychedelic clinical trial contexts, Richard and Levine propose utilizing the Microskills paradigm (i.e., a hierarchical categorization framework from the counseling literature of behaviors that occur in dyadic counseling interactions) [20,21]. The authors specify that “listening sequence” behaviors—which involve reflecting feelings, accurate summary reflections, attentive listening—constitute interpersonal interactions labeled as psychological support and “influencing” behaviors—involving teaching coping skills, reframing, providing directives—are indicative of interpersonal interactions that would be labeled as psychotherapy. The distinction offered by Richard and Levine is helpful insofar as the “microskills” are based on observable behaviors which would theoretically allow for more reliable evaluation of facilitator adherence to the intended interpersonal interaction. While a helpful contribution, we suggest that Richard and Levine's definition of psychological support could be improved conceptually by emphasizing the purpose of the defined interpersonal interaction and specifying psychological support in such a way that it can be functionally applied in clinical trials. This is important, as their current model may be difficult to implement in these settings given that it is not sensitive to the need to apply “influencing” behaviors in certain contexts in order to maintain safety: for example, crisis management and prevention, consenting to repeated safety-related blood pressure readings, maintaining ethical standards (e.g., the provision of ongoing informed consent), and directing participants through study protocols (e.g., directing participants to remain in a prone position, directing participants to avoid consuming alcohol or drugs on dosing days).

We argue that for psychological support to be of practical use in helping to disentangle classic psychedelic drug effects from psychotherapeutic effects, it must be defined in such a way that it is directed solely towards safety and viable for use within real-world research and clinical contexts. Moreover, given that interpersonal interactions have been inconsistently labeled and applied in psychedelic research, psychological support and psychotherapy must be clearly defined and reliably distinguished from each other. Once these goals are achieved, we believe valid empirical tests evaluating the role and necessity of

² We are sympathetic to other scholars' arguments about the unavoidable subtle influence of setting (e.g., therapy team) on psychedelic drug experience and clinical response, yet still believe meaningfully mitigating the interpersonal interaction's confounding effect on efficacy is attainable.

³ According to Kirlić et al. [35], the interpersonal interaction of the COMP360 trial included the following specific interventions: Discussing psychoeducation material and exploring expectations; Discussing key life events; Setting an intention for administration; Establishing support strategies for administration; Establishing boundaries around physical touch; Practicing experiential exercises; Reconfirming participant's intention for administration; Reconfirming support strategies for administration; Implementing support strategies as needed

and agreed upon; Selecting salient themes; Exploring salient themes and making meaning with nondirective support from the therapist; Identifying activities for ongoing integration outside of sessions [35]

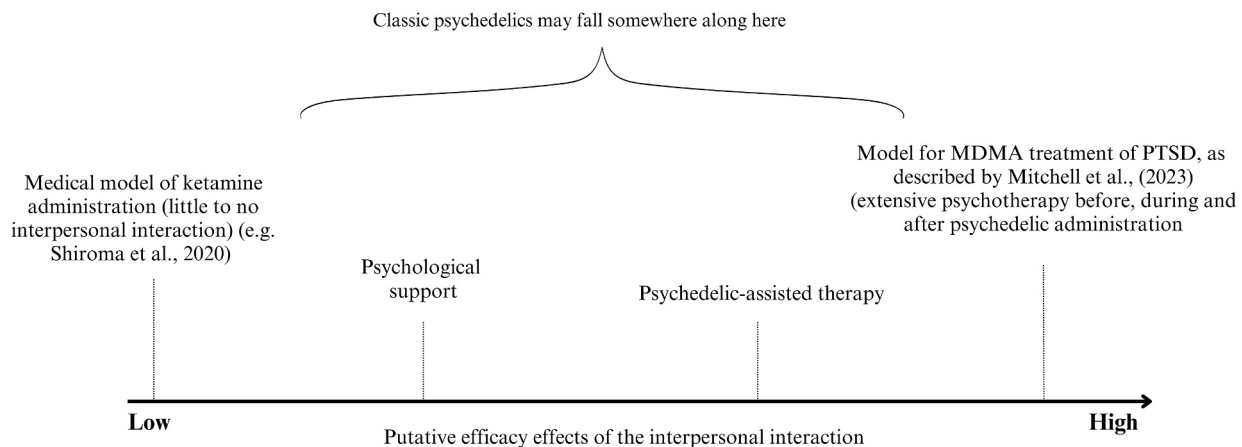


Fig. 1. Putative Efficacy Effects of the Interpersonal Interaction [18,19].

psychotherapy in psychedelic medicine can and should be attempted.

To that end, this paper has five specific aims: 1) propose a clear definition with observable criteria for psychotherapy within the context of classic psychedelic administration contexts (i.e., 5-HT_{2A} partial agonists such as psilocybin, LSD, DMT, 5-MeO-DMT, and mescaline; referred to as "psychedelics" hereafter), 2) propose a clear definition with observable criteria for psychological support in the context of classic psychedelic administration contexts, 3) propose and trial a coding procedure for reliably distinguishing psychological support from psychotherapy approaches, as defined here, 4) propose practical strategies for implementing these interpersonal interactions in psychedelic administration contexts, and 5) discuss empirical tests that could be used to evaluate relative safety and efficacy of the above interpersonal interactions. The present aims are intended to delimit psychotherapy from psychological support for scientific and clinical purposes, namely the scientific purpose of drawing causal inference about the therapeutic action of psychedelic compounds while controlling as best as possible for the confounding efficacy of the interpersonal interaction; and the clinical purpose of providing a means, via protocol standardization, of clearly delineating the additive efficacy of psychotherapeutic methods beyond psychedelic intervention.⁴

1.1. Defining psychotherapy

The development of a consensus definition of psychotherapy has been elusive historically [22], and will likely remain so. Our goal here is to define psychotherapy in such a way that the defining criteria are observable, relevant to psychedelic drug administration contexts, and shared across all of the below selected influential definitions of psychotherapy in the fields of clinical psychology and psychiatry. The American Psychological Association (APA) adopted the following definition of psychotherapy:

⁴ The present aims are expressly not intended to arbitrarily remove a supportive framework from psychedelic treatment, and the present authors support complementary efforts to identify efficacious psychosocial adjuncts that maximize patient benefit within clinical contexts. Indeed, it bears emphasizing the present authors' openness to future findings in line with the critical importance of psychotherapy and the inadequacy of a more narrowly defined, safety-focused form of psychological support, should future research support this conclusion. In fact, a failure to study psychedelic treatment without psychotherapy could inadvertently cause harm to future patients. If psychotherapy is in fact necessary for ensuring the safety and efficacy of psychedelic therapy, but its absence is never systematically studied, then regulatory agencies could conceivably approve the medical use of psychedelic treatments without insisting on a Risk Evaluation and Mitigation Strategy that enforces adequate psychosocial support, thereby failing to protect patients.

"psychotherapy is the informed and intentional application of clinical methods and interpersonal stances derived from established psychological principles for the purpose of assisting people to modify their behaviors, cognitions, emotions, and/or other personal characteristics in directions that the participants deem desirable." [23].

Norcross's definition of psychotherapy that the APA draws upon attempts to be broad enough to satisfy the eclectic nature of the field while still establishing some conditions around the concept. This definition appears to center the *intention* to treat psychological concerns using interventions derived from "established" psychological principles. Note, however, that the method by which these principles are established is not explicitly stated here, perhaps so as not to exclude therapeutic interventions or modalities that are derived from psychological principles that have not yet been the subject of rigorous scientific inquiry.

Alternatively, the European Association for Psychotherapy (EAP) adopted the following definition of psychotherapy [24]:

"The practice of psychotherapy is the comprehensive, conscious and planned treatment of psychosocial, psychosomatic and behavioral disturbances or states of suffering with scientific psychotherapeutic methods, through an interaction between one or more persons being treated, and one or more psychotherapists, with the aim of relieving disturbing attitudes to change, and to promote the maturation, development and health of the treated person. It requires both a general and a specific training/education."

Similar to the APA's characterization, the EAP definition emphasizes the importance of the intention to treat using psychological methods, but places more focus on the relationship between the therapist and the patient and explicitly states that the psychological methods utilized are derived from science and delivered by a person who has received training/education.

In Frank and Frank [25] seminal book, "*Persuasion and Healing: A Comparative Study of Psychotherapy*," the authors define it by the following three characteristics: 1) a setting and conceptual framework that specifies a relationship between a therapist and a patient, 2) the deliberate intention to alleviate a patient's demoralization (i.e., a sense of powerlessness to change oneself or one's environment), and 3) the use of rhetorical methods to persuade the patient to change the meaning that they apply to their symptoms and instill hope that they can overcome these symptoms [25]. Frank and Frank's definition centers the importance in psychotherapy of providing the patient a persuasive rationale that both explains their symptoms and offers plausible methods to improve them. This model is broad and suggests that there are common factors underlying all psychotherapies, which has been discussed in the context of psychedelic treatments [26,27].

Although the definitions provided above differ slightly in their emphasis, they do point to some common elements or factors that may be considered core features of psychotherapy. Wampold evaluated the empirical evidence for “common factors” in psychotherapy and found that the most well-studied common factors featured in all psychotherapies include 1) the establishment and maintenance of working alliance (i.e., the interpersonal relationship between therapist and client as well as the agreement between the therapist and client about the goals and tasks of therapy), 2) the therapist's demonstration of both cognitive and emotional empathy towards the client, and 3) the therapist's introduction of expectations (i.e., provision of a rationale for how and why the therapy being offered can be helpful for the client's specific presenting problem). The contextual model is another common factors model that is often referenced, and emphasizes the relationship, the creation of expectations through the explanation of disorder and treatment involved, and the performance of health promoting actions [22,28].

Overarchingly, when considering both the empirically-derived common features of psychotherapy and the different conceptual definitions of psychotherapy presented above, four common components of psychotherapy appear to emerge, namely: 1) the therapist's application of *psychological methods* (i.e., interventions that are delivered via listening, talking, non-verbal communication, interpersonal stances, or application of relevant knowledge), 2) the therapist's *intention* to intervene on psychological processes related to the patient's presenting psychological problem, 3) communication of a *rationale* for how the therapist's psychological methods will improve the patient's presenting psychological problem, and 4) the directed delivery of these psychological methods within the context of a *working alliance* between the patient and therapist to intervene on a psychological process related to the presenting psychological problem. Consistent with this finding, we define psychotherapy as an intervention that meets all four of the above criteria. This definition includes all psychotherapies of which we are aware while excluding other types of therapy (e.g., pharmacotherapy, physical therapy, massage therapy, bibliotherapy) and other forms of psychosocial support (e.g., pre-operation conversation with a nurse before a surgical procedure, supportive conversation with a family member, empathetic conversation with a hair stylist, or coaching).⁵

1.2. Psychotherapy in the context of psychedelic medicine

Congruent with the definition of psychotherapy presented above, we argue that psychotherapy in psychedelic administration contexts is an interpersonal interaction directed towards both safety as well as efficacy and contains the elements of: 1) *psychological methods* (i.e., *interventions that are delivered via listening, talking, non-verbal communication, interpersonal stances, or application of relevant knowledge*), 2) *intention-to-treat* (i.e., *intervening on a psychological process related to the participant's presenting psychological problem using psychological methods*),⁶ 3) *rationale* (i.e., *the provision of an argument that attempts to justify the use and/or explain the therapeutic value of the psychological methods used in the treatment*), and 4) *working alliance* (i.e., *a relationship between the facilitator and participant in which both parties collaborate to reach agreement on*

⁵ Note, our definition of psychotherapy does not require that the psychological methods that are applied are empirically validated nor does it specify that the rationales offered are based on established psychological science. The reason for this is that practitioners and researchers of psychotherapy have not reached a consensus on what constitutes established methods or rationales and it is beyond the scope of this paper to set parameters around what constitutes an established psychological method or rationale.

⁶ Psychological methods that are not needed to preserve ethical standards/participant safety nor explicitly intended to treat the participant's presenting problem, but that at some substantive probability engage related psychological processes are considered to meet this criterion, e.g., encouragement of participants' reflection on meaningful themes from their psychedelic experience.

the goals and/or tasks of treatment).⁷ We argue that interpersonal interactions that do not contain all four elements should not be categorized as psychotherapy, but could potentially be characterized as psychological support or some other type of interpersonal interaction.

1.3. Defining psychological support in the context of psychedelic medicine

As we have suggested above, psychological support has yet to be defined in such a way that it is directed solely towards participant safety and is viable within real-world psychedelic administration contexts. To address this issue, we propose that *psychological support* be defined as an interpersonal interaction that meets the following five criteria: 1) facilitator applies psychological methods (i.e., interventions that are delivered via listening, talking, non-verbal communication, interpersonal stances, or application of specialized knowledge), 2) facilitator intends to intervene on psychological processes only as necessary to maintain the participant's safety and adhere to ethical standards in psychedelic drug administration contexts, 3) facilitator explicitly avoids *intention* to use psychological methods to intervene on the participant's presenting psychological problem or related psychological processes, 4) facilitator explicitly avoids articulating or implying a rationale indicating that psychological methods are an active component of treatment for the participant's presenting psychological problem or related psychological processes, and 5) the facilitator establishes rapport with the participant with the goal of maintaining participant safety and meeting ethical standards in clinical research settings.

The psychological support definition proposed above is intended to specify an interpersonal interaction that can be readily standardized, reliably distinguished from psychotherapy, viable within psychedelic administration contexts, and limits the putative treatment effects attributable to the interpersonal interaction as much as is realistically attainable while preserving an intervention that has the capacity to adequately maintain participant safety (for a summary of the putative efficacy effects of each mode of interpersonal interaction, see Fig. 1, “Putative Efficacy Effects of the Interpersonal Interaction”). We acknowledge that psychological support, as defined here, may have some unintended psychotherapeutic effects (i.e., effects on efficacy), but argue that defining psychological support in an alternative way that somehow eliminates all potentially therapeutic effects is either impractical, unsafe, or unethical in psychedelic administration contexts.

1.4. Five criteria for psychological support

Below, we further describe each of our five proposed criteria for psychological support and differentiate our criteria for psychological support from psychotherapy, where appropriate (for a summary of these criteria, see Fig. 2, “Criteria for Psychological Support and Psychotherapy”).

(1) *Application of Psychological Methods*. Psychological methods are broadly defined here as all non-pharmacological and non-kinetic methods available to people to engage with and influence other people. Fundamentally, these methods are delivered via expressive verbal communication (e.g., talking, writing), receptive verbal communication (e.g., listening), non-verbal communication (e.g., facial expressions, body language), acquisition and application of relevant knowledge (e.g., understanding how psychological interventions work, cultural competence, obtaining a participant's biopsychosocial history, gathering data obtained from psychometric inventories), and interpersonal stances (e.g., the therapist/client or facilitator/participant interpersonal dynamic). The facilitator's application of psychological methods is a common

⁷ Collaborative selection of different tasks includes not only selecting different therapeutic techniques (e.g., cognitive restructuring), but also selecting different goals in the context of applying a singular technique (e.g., restructuring esteem beliefs versus intimacy beliefs).

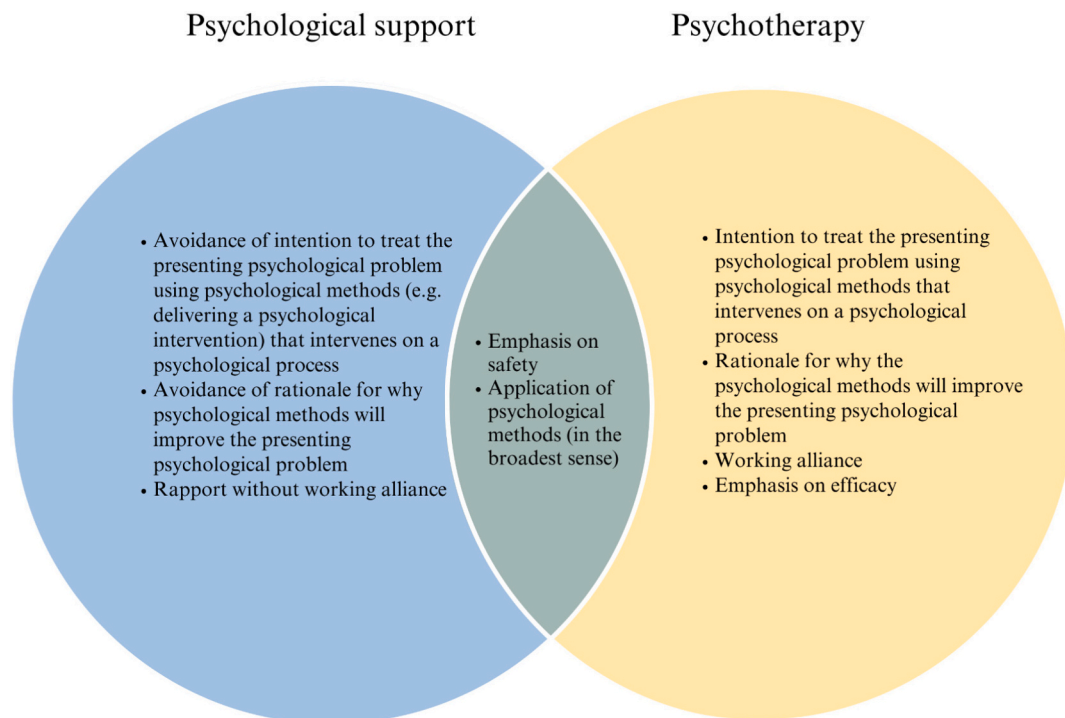


Fig. 2. Criteria for Psychological Support and Psychotherapy.

feature of both psychotherapy and psychological support.

(2) *Directed Solely towards Safety.* In a psychological support model, the psychological methods applied by the facilitators must be directed solely towards maintaining participant safety. This is distinct from psychotherapy models in which facilitators may direct psychological methods towards maintaining safety and/or enhancing efficacy. Notably, it is not relevant here whether a facilitator is engaging in a directive (i.e., structured and facilitator-led) or non-directive (i.e., unstructured and participant-led) process when applying psychological methods; rather, the critical determinant is whether the facilitator applies psychological methods with the intent to maintain participant safety (permissible in a psychological support model) or applies psychological methods with the intent to accomplish some other objective (generally not permissible in a psychological support model; see below for exceptions). Given the singular safety focus of psychological support, facilitators would be familiar with the safety concerns that may arise during psychedelic drug administration, so that they have a plan for preventing and managing them. These safety concerns, although very rarely encountered in research settings, include suicidal ideation, homicidal ideation, non-suicidal self-injurious behavior, physical endangerment (e.g., attempting to flee the drug administration setting while experiencing acute drug effects), life threatening medical events (e.g., extreme hypertension), and the emergence of psychotic symptoms [29]. Facilitators operating within this framework would endeavor to deploy the minimum amount of directed psychological methods necessary to prevent and manage the above safety concerns. This is inclusive of psychological methods that may also inadvertently enhance efficacy and even frank psychotherapeutic interventions that are known to enhance efficacy; so long as a clear and compelling argument can be made that the intervention is needed to ensure participant safety (as defined above), that intervention is allowable. In a study context in which the proposed psychological support model is meant to be technically applied, the administration of such psychotherapeutic procedures would need to be documented and reported.

Psychological methods directed towards safety in a psychological support model may be either preventative or reactive. Preventative measures include maintaining ongoing informed consent, obtaining

participants' relevant biopsychosocial history prior to drug administration, visual monitoring of the participant during drug administration with occasional verbal check-ins, and proactively assessing for adverse events and safety concerns after drug administration. Ongoing informed consent, which is inclusive of psychoeducation about likely psychedelic effects, is hypothesized to be highly relevant to the preservation of safety, as participants' sense of safety before, during, and after drug administration is likely to be enhanced if they are fully aware of the potential subjective and physical effects of the psychedelic compound being administered [30]. For example, participants who are unaware that psilocybin can cause feelings of nausea [29] might be more likely to appraise such sensations as threatening or harmful than participants who are informed that these sensations can occur. Similarly, facilitators may also offer brief guidance on how participants might best respond to distressing effects caused by the psychedelic compound itself that might occur in drug administration sessions. If guidance of this type is presented to participants, it would need to be protocolized and facilitators would clearly state to participants that the guidance is intended solely to help manage the unpleasant acute effects of the psychedelic compound and is not intended to treat the participant's presenting problem or any other mental health condition or symptom. Although psychoeducation about likely psychedelic effects and guidance around how to manage these effects are necessary components of psychological support, psychoeducation about the presenting psychological problem being studied (e.g., psychological theories about etiology, symptom maintenance, or treatment approaches) can affect efficacy [31] and would be avoided unless deemed absolutely necessary to ensure safety.

Reactive safety measures may include the use of any psychological methods necessary to maintain participant safety (e.g., reassurance, use of grounding techniques, providing psychoeducation about symptoms, alerting appropriate authorities). In the rare event that safety concerns arise that cannot be managed using psychological methods alone, facilitators should be prepared to contact a study physician to administer "rescue medication" to reduce psychological distress to manageable levels (e.g., benzodiazepines), terminate the psychedelic experience (e.g., atypical antipsychotics), or treat cardiovascular events (e.g., an antihypertensive). In a psychological support model, facilitators would

be expected to refer participants to appropriate external psychotherapeutic or psychiatric resources (e.g., emergency department, partial or inpatient psychiatric hospitalization) in the rare event that these referrals are needed to maintain or preserve participant safety.

In accordance with the sole focus on safety, treatment protocols that purport to use a psychological support model would limit participant interactions and encounters with facilitators to those that are necessary to ensure a reasonable degree of participant safety. In other words, unless a compelling argument can readily be made as to why an interaction or encounter is necessary due to safety considerations, the encounter would not be included in the study protocol. For this reason, it is expected that the interpersonal interaction in psychological support protocols will be more limited in duration and number of encounters than it is in psychotherapy protocols.

The sole emphasis on safety will likely have the largest practical impact on what is commonly referred to as the “integration” phase of the interpersonal interaction (i.e., the phase that occurs after the acute effects of the drug have subsided), as this is the phase in which psychotherapy, as defined here, is most likely to occur. The term “integration” itself may imply to some facilitators and participants that a psychotherapeutic process will occur, even if the application of psychotherapy is not explicitly stated in the treatment protocol. For this reason, it is recommended that in a psychological support model, these post-drug administration encounters be labeled as “safety monitoring visits” or some other term that clearly indicates that the goal of these encounters is to maintain safety rather than to engage in psychotherapeutic processes. To monitor safety effectively during these visits, facilitators in a psychological support model can assess safety concerns directly by conducting a structured safety and adverse events assessment. Facilitators may also be permitted to assess for safety in a more unstructured way (e.g., listening to the participant share their account of their psychedelic experience) while actively listening for safety concerns.

Finally, psychological methods directed towards maintaining ethical standards in psychedelic administration contexts (e.g., obtaining consent, providing information about procedures, ensuring basic participant physical comfort during drug administration by providing drinking water, comfortable seating, adjusting climate controls as needed), though not required to maintain safety, are allowable in a psychological support model. These are basic standards for all manner of social roles, including research assistants and supportive personnel across a wide range of research and clinical contexts far beyond psychotherapy and psychedelic contexts.

(3) *Explicit Avoidance of Intention to Treat.* Consistent with psychological support's sole focus on safety, facilitators would avoid engaging in directed psychological methods (e.g., deploying psychological interventions) with the intent to treat the participant's presenting psychological problem or related psychological processes. This, of course, differs substantially from a psychotherapy model in which an explicit intention to treat the participant's presenting psychological problem would be present. The directive to avoid intention to treat, if adhered to, would plainly prohibit the facilitator's application of any formal, evidence-based psychotherapy packages (e.g., acceptance-commitment therapy, exposure and response prevention) and structured psychological interventions (e.g. interoceptive exposure exercises, cognitive

restructuring), but would also discourage the facilitator's application of myriad psychological methods that they might otherwise use with the intent to treat the participant's presenting problem (e.g., reassuring a depressed participant about their career prospects, exploring the participant's values, setting a plan with the participant to build on presumably therapeutic insights developed through the psychedelic experience, setting a therapeutic intention for the psychedelic experience, suggesting a meditation practice).⁸

In addition to avoidance of intention to treat the participant's presenting psychological problem, facilitators would explicitly state that their goal is solely to ensure safety during the participant's experience in the psychedelic treatment protocol, *not* to provide psychotherapy for the participant's presenting problem. Facilitators would be expected to explicitly communicate their intended function to participants by administering a role induction as part of informed consent that clarifies their role in the study and offers participants an opportunity to ask questions about the facilitator's role. It is recommended that the facilitator also remind the participant of their role throughout the study protocol, as needed. The facilitator's explicit avoidance of intention to treat serves a triple function: 1) it manages the participant's expectations in a way that might limit expectancy effects attributable to the interpersonal interaction, 2) it clarifies the facilitator's role such that participants will be less likely to attempt to engage in psychotherapy with facilitators, and 3) facilitators may themselves feel less pull to provide psychotherapy, which could potentially enhance adherence to the psychological support model.

(4) *Explicit Avoidance of Rationale.* In a psychological support model, as defined here, the psychedelic compound itself is explicitly presented as the sole active treatment being studied. Therefore, the facilitator would *not* articulate (i.e., offer a rationale) or otherwise imply to the participant that psychological methods deployed by the facilitator during the psychedelic treatment protocol are, or may be, efficacious in treating the participant's presenting psychological problem or related psychological processes. In accordance with this guidance, the facilitator would also refrain from introducing psychotherapeutic theories, even in an off-hand manner, about the maintenance or amelioration of psychiatric disorders during their interactions with participants. Rather, the facilitator is strongly encouraged to explicitly state the opposite to the participant (i.e., that any psychological methods employed by the facilitator are not intended to treat the participant's presenting problem and are not an active ingredient of the psychedelic treatment protocol). This communication may help to further reduce unwanted expectancy effects attributable to the interpersonal interaction, particularly since there is evidence that the effectiveness of psychotherapy itself for many mental health conditions may be less attributable to the application of specific techniques than the ability of the therapist to offer a persuasive rationale as to why the techniques they use, whatever they may be, are efficacious [28].

Notably, the facilitator in a psychological support model is permitted to offer rationale to the participant as to how the psychedelic compound might affect psychological problems and processes (e.g., enhanced neurological plasticity), provided that they frame these psychological effects as dependent solely upon drug effects rather than psychological methods deployed by the facilitator.

⁸ Certain psychological methods that are not needed to preserve ethical standards/participant safety nor explicitly intended to treat the participant's presenting problem or related psychological processes can nevertheless be reasonably expected to contribute to efficacy at some substantive probability. One common example includes creating space in integration sessions for the participant's contemplation of meaningful themes from their psychedelic experience. Such methods are considered to contravene *explicit avoidance of intention to treat* in view of their probabilistic effect on efficacy and their probabilistic engagement with psychological processes related to the participant's presenting problem.

(5) *Rapport*. The facilitator is encouraged to establish a good working rapport with the participant such that they can effectively maintain participant safety and guide the participant through the treatment protocol. Rapport is distinguishable from the working alliance that is an integral component of psychotherapy, in that working alliance requires that the facilitator and participants engage in a collaborative process to agree on the goals and/or tasks of the treatment. This collaborative process is not needed to establish a rapport with a participant, and more importantly, would be undesirable in a psychological support model where the treatment tasks (i.e., administration of a psychedelic compound) and goal (i.e., treatment of the presenting psychological problem being studied using the psychedelic compound) are intended to be uniform across all participants.

Rapport can be established and maintained by adopting the following approaches when interacting with participants: 1) demonstrating professionalism, reliability, and empathy in interactions with the participant, 2) upholding relevant ethical standards (e.g., maintaining participant confidentiality, maintaining ongoing informed consent, avoiding any behaviors that are abusive or exploitative, adhering to the treatment protocol), 3) demonstrating an interest in the

participant's life story, their motivations for participating in the study, and the participant's experience(s) taking the psychedelic, and 4) maintaining a non-judgmental stance towards the participant.

Although this approach to developing rapport may elicit therapeutic effects that are not solely attributable to the psychedelic compound, the guidance above is congruent with ethical and professional standards of communication with participants across virtually all clinical trials and human subjects research contexts. Requiring facilitators not to adhere to these ethical communication standards in an attempt to eliminate all non-pharmacological impacts on efficacy is not only unethical, but may adversely impact safety. For example, consider how willing a participant would be to share information relevant to their safety if they were interacting with a facilitator who was instructed to intentionally be unprofessional, unreliable, callous, unethical, uninterested in them, and judgmental.

1.5. Application of proposed criteria to test cases

Although we have proposed our definitions/criteria with the ultimate aim of using them prospectively to develop psychological support

Table 1

Summary of labels used for interpersonal interaction in clinical trials examining the safety and/or efficacy of psychedelics for major depressive disorder, including treatment-resistant depression.

Study	Drug and Condition	Label applied by original study authors for the interpersonal interaction	Label according to our proposed criteria
Carhart-Harris, R. L., Bolstridge, M., Rucker, J., Day, C. M., Erritzoe, D., Kaelen, M., ... & Nutt, D. J. (2016). Psilocybin with psychological support for treatment-resistant depression: an open-label feasibility study. <i>The Lancet Psychiatry</i> , 3(7), 619–627.	Psilocybin for treatment-resistant depression	“Psychological support”	Clear psychotherapy
Sanches, R. F., de Lima Osório, F., Dos Santos, R. G., Macedo, L. R., Maia-de-Oliveira, J. P., Wichert-Ana, L., ... & Hallak, J. E. (2016). Antidepressant effects of a single dose of ayahuasca in patients with recurrent depression: a SPECT study. <i>Journal of clinical psychopharmacology</i> , 36(1), 77–81.	Ayahuasca for treatment-resistant depression	“No psychological intervention”	Probable psychological support
Palhano-Fontes, F., Barreto, D., Onias, H., Andrade, K. C., Novaes, M. M., Pessoa, J. A., ... & Araújo, D. B. (2019). Rapid antidepressant effects of the psychedelic ayahuasca in treatment-resistant depression: a randomized placebo-controlled trial. <i>Psychological medicine</i> , 49(4), 655–663.	Ayahuasca for treatment-resistant depression	“Support”	Indeterminate
Carhart-Harris, R., Giribaldi, B., Watts, R., Baker-Jones, M., Murphy-Beiner, A., Murphy, R., ... & Nutt, D. J. (2021). Trial of psilocybin versus escitalopram for depression. <i>New England Journal of Medicine</i> , 384(15), 1402–1411.	Psilocybin for depression	“Psychological support”	Clear psychotherapy
Davis, A. K., Barrett, F. S., May, D. G., Cosimano, M. P., Sepeda, N. D., Johnson, M. W., ... & Griffiths, R. R. (2021). Effects of psilocybin-assisted therapy on major depressive disorder: a randomized clinical trial. <i>JAMA psychiatry</i> , 78(5), 481–489.	Psilocybin for major depressive disorder	“Supportive psychotherapy”	Probable psychotherapy
D'Souza, D. C., Syed, S. A., Flynn, L. T., Safi-Aghdam, H., Cozzi, N. V., & Ranganathan, M. (2022). Exploratory study of the dose-related safety, tolerability, and efficacy of dimethyltryptamine (DMT) in healthy volunteers and major depressive disorder. <i>Neuropsychopharmacology</i> , 47(10), 1854–1862.	Dimethyltryptamine (DMT) in healthy volunteers and for major depressive disorder	“Psychological support”	Clear psychological support
Goodwin, G. M., Aaronson, S. T., Alvarez, O., Arden, P. C., Baker, A., Bennett, J. C., ... & Malievskaia, E. (2022). Single-dose psilocybin for a treatment-resistant episode of major depression. <i>New England Journal of Medicine</i> , 387(18), 1637–1648.	Psilocybin for a treatment-resistant episode of major depression	“Psychological support”	Probable psychotherapy
Raison, C. L., Sanacora, G., Woolley, J., Heinzerling, K., Dunlop, B. W., Brown, R. T., ... & Griffiths, R. R. (2023). Single-dose psilocybin treatment for major depressive disorder: a randomized clinical trial. <i>Jama</i> , 330(9), 843–853.	Psilocybin for major depressive disorder	“Psychological support”	Probable psychotherapy
von Rotz, R., Schindowski, E. M., Jungwirth, J., Schuldt, A., Rieser, N. M., Zahoransky, K., ... & Vollenweider, F. X. (2023). Single-dose psilocybin-assisted therapy in major depressive disorder: A placebo-controlled, double-blind, randomized clinical trial. <i>EClinicalMedicine</i> , 56.	Psilocybin for major depressive disorder	“Psychological support”	Probable psychotherapy
Sloshower, J., Skosnik, P. D., Safi-Aghdam, H., Pathania, S., Syed, S., Pittman, B., & D'Souza, D. C. (2023). Psilocybin-assisted therapy for major depressive disorder: An exploratory placebo-controlled, fixed-order trial. <i>Journal of Psychopharmacology</i> , 37(7), 698–706.	Psilocybin for major depressive disorder	“Psychotherapeutic support”	Clear psychotherapy
Rosenblat, J. D., Meshkat, S., Doyle, Z., Kaczmarek, E., Brudner, R. M., Kratiuk, K., ... & McIntyre, R. S. [24]. Psilocybin-assisted psychotherapy for treatment resistant depression: A randomized clinical trial evaluating repeated doses of psilocybin. <i>Med</i> , 5(3), 190–200.	Psilocybin for treatment-resistant depression	“Psychotherapy”	Probable psychotherapy

protocols which can be reliably distinguished from psychotherapy protocols and to develop adherence measures to evaluate fidelity to either model in future psychedelic clinical trials, we wanted to first trial the applicability of our criteria and definitions on published descriptions of interpersonal interactions used in psychedelic clinical trials to illustrate how psychotherapy and psychological support can be distinguished in practice. To accomplish this objective, we apply our proposed criteria to the interpersonal interaction described in all extant clinical trials examining the effects of psychedelics on major depressive disorder (see Table 1).

2. Method

A search was performed in the PubMed database from November to December 2024 for peer-reviewed publications of clinical trials in which the safety and/or efficacy of a classic psychedelic was examined in a population of patients with major depressive disorder or treatment-resistant depression. The primary search terms used were “classic psychedelic” (further specified by “psilocybin”, “LSD”, “ayahuasca” and “DMT”), “major depressive disorder”, and “treatment-resistant depression”. Studies published before 2006 were excluded for relevance.

To reach consensus on the final disposition label for each study, two independent raters (Rater #1 and Rater #2) first reviewed the methods section and related supplemental materials of each study publication to determine whether the interpersonal interaction described in the study met or probably met our proposed criteria for psychotherapy, met or probably met our proposed criteria for psychological support, or failed to meet our proposed criteria for either type of interpersonal interaction.

Raters were instructed to rate each proposed criterion for psychotherapy and psychological support as either “present,” “absent,” or “indeterminate.” A criterion was rated as “present” if the rater observed that the criterion was explicitly indicated as present or implied to be present. A criterion was rated as “absent” if the rater observed that the criterion was explicitly indicated as absent or implied to be absent. A criterion was rated as “indeterminate” if the rater could not determine whether the criterion was absent or present. These determinations were made solely from the written manuscript.

Raters were instructed to label the interpersonal interaction as “clear psychotherapy” if all four of the proposed criteria for psychotherapy are present. Raters were instructed to label the interpersonal interaction as “clear psychological support” if all five of the proposed criteria for psychological support are labeled as “present.” If full criteria for psychotherapy are not met, but at least two of the criteria for psychotherapy are labeled as “present” and none of the criteria for psychotherapy are labeled as “absent,” the raters were instructed to label the interpersonal interaction as “probable psychotherapy.” If full criteria for psychological support are not met, but at least two of the criteria for psychological support are labeled as “present” and none are labeled as “absent,” the raters were instructed to label the interpersonal interaction as “probable psychological support.” If all criteria for both psychological support and psychotherapy were labeled as “absent,” raters were instructed to label the interpersonal interaction as “neither.” Finally, raters were instructed to label interpersonal interactions that do not qualify for any of the above labels as “indeterminate.” Inter-rater reliability between the raters was measured by calculating a Cohen's Kappa (κ) coefficient.

To then reach consensus on a final disposition label for each study, any disagreement between the raters was then resolved via a mediation process, with the goal of identifying reason(s) for any observed inter-rater disagreement. This was moderated by a third member of the study team.

3. Results

In total, eleven studies met our inclusion criteria. These studies examined a range of classic psychedelics: eight focused on psilocybin, two on ayahuasca, and one on DMT. Based on our criteria, three studies

were classified as involving clear psychotherapy, five as probable psychotherapy, one as clear psychological support, one as probable psychological support, and one as indeterminate (see Table 1 for the labels applied to each study). Notably, two studies that the original study authors described as providing “psychological support” were rated as clear psychotherapy by our criterion [32,33]. According to the criteria set out by Landis and Koch [34], agreement between the raters was moderate (Cohen's Kappa of 0.52).

A mediation process then occurred. We began by noting that 68 % (17 out of 25 ratings) of the disagreement involved an “indeterminate” rating, while only 32 % (8 out of 25 ratings) of the disagreement involved present vs. absent ratings. It was discovered during the mediation process that the raters differed systematically with respect to their threshold for rating the implied presence or absence of a criterion, with one rater (Rater #1) more readily identifying implied presence or absence of criteria and rating accordingly while the other rater (Rater #2) took a more tentative approach and tended to rate criteria as “indeterminate.” Rater #2 accounted for 79 % (26 out of 33 ratings) of applied “indeterminate” ratings. In addition, the mediation discussion revealed that one rater (Rater #2) applied a more narrow interpretation of the *working alliance* and *rappport* criteria in their initial ratings than the other rater (Rater #1). Specifically, the raters differed in their interpretations of what degree of latitude for collaborative selection of psychological methods would constitute *working alliance*. Disagreement on these specific criteria accounted for 48 % (12 out of 25 ratings) of total disagreement between raters and 63 % (5 out of 8 ratings) of disagreement between raters that did not involve an “indeterminate” rating. After reaching consensus on the definitions of *working alliance* and *rappport* described in the manuscript and identifying a shared threshold for whether a criterion was implied to be present or absent, the raters were able to reach full convergence on criterion ratings and disposition of the interpersonal interaction (i.e., whether the interpersonal interaction described in the study publication was more consistent with our proposed definition of psychological support or our proposed definition of psychotherapy).

Finally, it should be noted that in the initial ratings there was no categorical disagreement (i.e., psychological support vs. psychotherapy) between raters on the disposition of the interpersonal interaction. Rather, initial disagreement was only observed with respect to the degree of certainty that the raters thought the described interpersonal interaction was consistent with psychological support or psychotherapy (e.g., indeterminate vs. probable psychological support, probable psychotherapy vs. clear psychotherapy). For more details on initial ratings, mediated ratings, and selected notes on the mediation process, see Supplemental Materials.

4. Discussion

We have proposed criteria-based definitions for the constructs of psychotherapy and psychological support within classic psychedelic drug administration contexts: both being frequently used but heretofore not well defined or clearly distinguishable from each other. We argue that the criteria-based definitions proposed here have the potential to allow for more reliable discrimination between these modes of interpersonal interaction. We performed a preliminary empirical test of this argument by using our criteria to rate the published description of the interpersonal interaction utilized in clinical trials examining the effects of psychedelics on major depressive disorder.

At the completion of the study rating process, the raters reached consensus that eight of eleven reviewed studies clearly or probably applied psychotherapy according to our definition, two studies clearly or probably applied psychological support according to our definition, and one study applied an interpersonal interaction that was rated as “indeterminate.” In addition, the label applied by the study authors to the interpersonal interaction differed categorically (i.e., psychotherapy vs. psychological support) from the label we applied to the interpersonal

interaction based on our criteria in five out of eleven studies.

Despite achieving full convergence on ratings at the conclusion of the rating process, the results of our initial ratings suggest that only a moderate level of inter-rater agreement could be achieved using our criteria. We argue that this finding is attributable to the ratings being based solely upon the published descriptions of the interpersonal interactions, which were often rather brief and vague. Consistent with this argument, two-thirds of initial inter-rater disagreement involved an “indeterminate” rating and the indeterminacy-related disagreement was attributable to the two raters having systematically different thresholds for determining whether a criterion was *implied* to be present or absent. Moreover, even after convergence between raters was reached, raters agreed that over 25 % of criterion ratings were “indeterminate,” suggesting continued uncertainty about the precise nature of the interpersonal method applied in the trial. We are confident that if our criteria were used to rate more detailed descriptions of the interpersonal interaction and/or we had access to more study materials pertaining to the interpersonal interaction (e.g., video recordings of facilitator-participant encounters, facilitator training manuals, protocol adherence measures, participant ratings of interactions with facilitators, informed consent forms), indeterminacy-based disagreement and/or the presence of indeterminate ratings would be substantially reduced or eliminated because raters would not need to rely on discerning implied presence or absence of criteria, but could instead simply observe the presence or absence of criteria by viewing the more comprehensive and richly detailed study materials. Finally, the moderate initial inter-rater agreement lends some empirical support to our and others' observation that the interpersonal interaction in psychedelic clinical trials has historically been imprecisely defined and poorly described [7]. Based on this finding, we suggest that psychedelic researchers publish more detailed descriptions of the interpersonal interactions used in clinical trials, so that the nature of the interpersonal interaction can be more easily evaluated and that study methods could be more readily reproduced. Moreover, given how imprecisely the interpersonal interaction has been described in psychedelic clinical trials, we argue that there is currently insufficient empirical basis for prescribing or recommending one type of interpersonal interaction over another.

Although there are extant examples of interpersonal interactions in the literature that most researchers would readily agree are psychological support (very minimal preparation and integration with few explicit protocolized psychological interventions) and others that most would readily agree are psychotherapy (explicitly using a manualized form of psychotherapy), there are many edge cases in which researchers disagree as to whether the interpersonal interaction is best characterized as psychological support or psychotherapy. We feel that the criteria proposed here could be used to more reliably categorize these edge cases into either psychological support or psychotherapy approaches, provided that the interpersonal interaction is well-described and/or relevant study materials (e.g., facilitator training materials, video/audio recordings of sessions) are made available for review. Of course, it is important that this claim be evaluated empirically, *such as by developing adherence measures based on our criteria for both types of interpersonal interactions and examining the inter-rater reliability of these measures in scenarios in which raters have full access to study materials relevant to the interpersonal interaction.*

Some may argue that drawing a distinction between psychotherapy and psychological support based on facilitator intent, as we have done here, rather than based solely on specific facilitator behaviors might adversely impact the development of an adherence measure with good inter-rater reliability. Although this is a valid critique, we chose to consider facilitator intent in our definitions because many specific behaviors that are needed to ensure participant safety and maintain ethical standards in psychedelic administration contexts may also have psychotherapeutic effects. We argue that because of this overlap, efforts to define psychological support based solely on specific behaviors alone will either be over-inclusive (i.e., psychological support is framed in

such a way that it is not meaningfully distinct from psychotherapy) or under-inclusive (i.e., psychological support is framed in such a way that it does not have the capacity to ensure participant safety or comply with ethical standards in clinical or research settings). Although we concede that intent is less readily observable than behavior, we argue that intent can be inferred in a sufficiently reliable way through considering a combination of observable facilitator behavior and the context in which that behavior occurs.

It is possible that some will not be convinced that the psychological support model proposed here is sufficiently non-psychotherapeutic for conceptual reasons (i.e., a less supportive interpersonal interaction than proposed here can be devised). Conversely, some may hold that psychological support, as defined here, is not sufficient to preserve safe psychedelic drug administration (i.e., a more supportive interpersonal interaction than proposed here is necessary). With respect to the former critique, we concede that our definition of psychological support is not devoid of all potentially psychotherapeutic elements; however, as discussed above, an interpersonal interaction that has been totally scrubbed of all potentially psychotherapeutic components is fundamentally unethical and unsafe in psychedelic clinical and research contexts, and therefore, we argue is an unattainable and undesirable goal. Regarding the latter critique, we argue that studying the proposed form of psychological support is a reasonable first step in the process of empirically evaluating whether a minimal form of interpersonal interaction can preserve safe psychedelic drug administration while remaining scientifically useful; adjustments may be needed.

Thus far, we have refrained from discussing specific skills, training, or credentials that facilitators of psychological support should possess, because we wanted to focus this paper on defining and describing the psychological support interaction itself rather than defining and describing the qualifications of the psychological support facilitator. We would argue, however, that mental health professionals (e.g., psychiatrists, psychologists, mental health counselors, social workers), particularly those who have psychotherapy training, would be well-suited to the role of facilitator, as they are most likely to possess the skills, training, and qualifications necessary to satisfactorily perform the primary function of psychological support (as defined here)—maintaining participant safety in psychedelic drug administration contexts.

Some might argue that psychotherapy will inevitably be applied if facilitators of psychological support are required to be psychotherapists, even if the facilitators are instructed to refrain from doing so. We argue the contrary. It is often those who are most skilled in a discipline that best understand the boundaries of that discipline, and therefore, would be least likely to inadvertently violate those boundaries. Consistent with this logic, we argue that psychotherapists would potentially perform much better than non-psychotherapists at adhering to a directive to refrain from applying psychotherapy. Moreover, psychotherapists who possess a background in psychological assessment might be particularly adept at providing psychological support, because they already have professional training and experience in refraining from offering psychotherapy during professional interactions, as the provision of psychotherapy is not only not required, but is also undesirable or even prohibited in psychological assessment contexts.

Others might argue that if psychological support is not psychotherapy, then there is no legitimate reason to require that facilitators be mental health professionals. This argument assumes that the task of ensuring participant safety in psychedelic drug administration contexts does not require, nor does it substantially benefit from, the skills, training, and qualifications that mental health professionals possess. We have argued above that mental health professionals would be well-suited to satisfactorily perform this task, but ultimately, whether psychological support must be administered by a mental health professional to maintain both an adequate safety profile and adherence to the psychological support model is an empirical question.

While much of what we have discussed above has been conceptual and some of this discussion will ultimately remain in this domain, there

are indeed empirical tests that can be conducted to address the aforementioned critiques as well as other important aspects of the ongoing debate about the role of psychotherapy in psychedelic medicine. It would be interesting and helpful to the debate if a psychedelic study experimentally manipulated the interpersonal interaction variable, by comparing one condition receiving clear psychological support and another condition receiving clear psychotherapy and comparing the resulting risk/benefit profile. This would at least provide data on the magnitude of the difference and thus inform the stakes of the current debate. If consensus is achieved on operationalizable definitions of psychological support and psychotherapy with adherence criteria, we argue that it would at last be possible to evaluate these and other important empirical questions.

Among the most effective study methodologies for evaluating these questions would be a two-by-two factorial design including the following four conditions—1) psychedelic with psychological support, 2) psychedelic with psychotherapy, 3) placebo with psychological support, and 4) placebo with psychotherapy. Such a study would help to elucidate the relative contributions of psychotherapy and psychedelics to safety and efficacy outcomes.

Researchers who are interested in this question, but are not positioned to conduct large experimental studies with multiple conditions could also gather relevant data by using alternative methodologies (e.g., conduct a meta-analysis comparing the effect sizes of psychedelic trials that apply psychological support to those that apply psychotherapy, conduct a non-inferiority trial with participants in one condition receiving psychotherapy and participants in another condition receiving psychological support). Of course, these alternative approaches would require the existence of (in the case of a meta-analysis) or the implementation of (in the case of a non-inferiority trial) study protocols that apply interpersonal interactions that meet our definitions of psychotherapy and/or psychological support.

A potential concern about psychological support is that the failure mode of psychological support is psychotherapy. That is, a facilitator may intend to administer psychological support but instead administer psychotherapy, either inadvertently or ostensibly for safety reasons. This is perhaps more likely with more hours of contact time, with more difficult psychedelic experiences, and with more complicated clinical conditions. Thus, adherence rating of psychological support will be key to demonstrate that psychological support, as defined here, was performed and that psychotherapy, as defined here, was not performed. To optimize inter-rater reliability, an adherence scale should include items that assess readily observable facilitator behavior, where feasible, as well as a mechanism for reliably assessing facilitator intent through a combination of facilitator behavior and the context in which the behavior occurs. Adherence scales could draw upon a variety of information sources to increase reliability and validity, including behavioral coding of sessions, participant self-report, facilitator self-report, and review of study materials that are relevant to the interpersonal interaction (e.g., facilitator training manuals). With respect to behavioral coding specifically, Microskills concepts discussed in Richard & Levine [21] could potentially be of value in categorizing specific facilitator behaviors as more or less consistent with either mode of interpersonal interaction, provided that the procedure developed for behavioral coding is sensitive to the important contextual and functional considerations provided here that we argue distinguish psychotherapy from psychological support. Without the development and implementation of well-designed adherence measures, it is likely that interventions putatively framed as psychological support will actually end up containing psychotherapy, thereby adversely affecting consistent application of the psychological support model and replicability of study findings.

5. Conclusion

We believe that the definitions and the discriminant criteria proposed here will provide tools to facilitate a more productive discussion

on the role of psychotherapy in psychedelic medicine. With shared definitions of psychotherapy and psychological support in hand, the interpersonal interactions that occur in psychedelic administration contexts may be more reliably characterized and more effectively studied. Here we aim to *describe these distinct approaches rather than prescribe the use of one over the other*. Researchers who are focused on examining the narrow efficacy of psychedelic drugs or the truly additive effects of psychotherapeutic adjuncts can empirically evaluate the relative efficacy of these interpersonal interactions in diverse clinical populations, avail themselves of a well-defined and potentially easily standardized psychosocial control condition that reduces psychotherapy (and its putative therapeutic effects) as much as is feasible in psychedelic drug administration contexts, and finally, begin to elucidate the precise therapeutic efficacy of psychedelic treatments with and without psychotherapy.

CRedit authorship contribution statement

Jonathan R. Schettino: Writing – review & editing, Writing – original draft, Conceptualization. **Katherine Cheung:** Writing – review & editing, Writing – original draft, Conceptualization. **Sandeep M. Nayak:** Writing – review & editing, Writing – original draft, Conceptualization. **Brandon M. Weiss:** Data curation, Methodology. **David B. Yaden:** Writing – review & editing, Writing – original draft, Conceptualization.

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Declaration of competing interest

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Appendix A. Supplementary data

Supplementary data to this article can be found online at <https://doi.org/10.1016/j.genhosppsych.2025.12.021>.

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